

SURF Fellowship Project

Selective Vulnerabilities: Stigma in Antimicrobial Resistance Narratives in the News

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Abstract

“One feature of the usual script for plague: the disease invariably comes from somewhere else.” (Sontag, *AIDS and Its Metaphors*). During COVID-19, the world witnessed how stigmatization of China, foreigners, and “others” worsened health inequalities and hindered public health. We risk repeating these mistakes just 5 years later, as similar narratives emerge in Antimicrobial Resistance (AMR)- when infections stop responding to antimicrobial drugs. By critically analyzing news as narrative spaces, this paper elucidates how the selective construction of vulnerability furthers health injustice by stigmatizing and failing to foster comprehensive engagement with this omnipresent health threat.

Using thematic and rhetorical analysis, I examine 363 AMR news articles from digital UK outlets to elucidate differences between how various countries and populations’ vulnerabilities are constructed. Articles invoking risk for UK and western populations focus on UTI’s and STD’s, which unnecessarily moralizes AMR and ties vulnerability to womanhood. Conversely, non-Western portrayals of AMR emphasize infrastructural poverty, unsanitary and wartime conditions in developing countries, creating uneven portrayals of blame and experience in these localities. Highlighting the most vulnerable is important to recognize diverse illness experiences and mobilize vital resources for treatment, but it risks harmful stigmatization. In this case, an inequitable distribution of the causes of AMR across localities and populations veers too

far, increasing stigma and eclipsing broader vulnerabilities. Microbes' disregard for borders and demographics demands that we address this problem for all— I call for new narratives, which foster broader equitable engagement, to be devised.

Conference Speech:

I'd like to start with a quote to get you situated: "One feature of the usual script for plague: the disease invariably comes from somewhere else." Susan Sontag wrote this in *AIDS and Its Metaphors*, demonstrating the persistent link between disease and that which is foreign. Consider the 1918 "Spanish Flu" which likely started in Kansas, but made it's way around the world with almost every country keeping it hush hush— until it got to Spain. They were willing to talk about it. Hence, we now call it the Spanish flu. We also saw this in action just a few years ago— COVID originated in China, and consequently stigmatizing language lingered around against Asian Americans, and we saw a massive rise in Asian hate crimes. This is because we understand the world through the stories we tell ourselves, and when those stories turn hateful, the outcomes can have real impacts. In this case, the stigmatizing language of infectious disease burdens individuals and leads to broader injustices— both in their health outcomes and more broadly.

Just a few years later, we risk making the same, stigmatizing mistakes as new infectious diseases emerge. In my research last summer, I analyzed how stigma emerged in the context of Antimicrobial resistance narratives, or AMR, the phenomenon where infections slowly stop responding or lose efficacy to the drugs meant to treat them. This is a major global problem, often unnoticed, but killing an estimated 1.14 million annually across the globe, demonstrating

its widespread impact. To better understand how this global problem takes shape when it is discussed in local contexts, I used thematic and rhetorical analysis to examine 363 AMR news articles from digital UK outlets, in 2024-2025, and found that there was a dramatic difference between how various countries and populations' vulnerabilities are constructed. I find that the figurative and descriptive language is common in non-western localities and minority patients, and cold statistics color Western and non-minority anecdotes, which creates uneven portrayals of blame and experience, or what I call selective vulnerabilities, that overly stigmatize. While they may be operating in an attempt to shed light in health inequalities– the language goes too far in its stigmatizing of the populations it represents. To ground what I'm saying in some real evidence, I'm going to show you some quotes from these news articles:

“Next epidemic to spill out of China could be SUPER GONORRHEA - where rate of antibiotic-resistant STIs are 40x higher than US and UK”

This, fairly self-evident title burdens China with the emergence of an imagined future pandemic that could emerge anywhere. It is equally rooted in stigma from COVID and perpetuates it. It suggests AMR is something that *spills*, like the infections are water in a steadily overflowing cup that is China, waiting any moment to hit– obscuring the fact that there's a spill in the west too– even though it's quite literally in the title. If you want to worry about Gonorrhea as a UK citizen, you should probably be worried about other people in the UK, just statistically speaking. Not only does this article fail to foster the comprehensive work needed both at home and abroad to mitigate the spread of AMR, but the creatively named “super gonorrhea” stigmatizes asian foreigners unnecessarily.

The next quote goes into great detail about a child from a war-torn region in Ethiopia with an AMR infection.

“Her tiny body hooked up to machines twice her size, her mother standing vigil at the side of her cot, Yusra was in a struggle for life... Doctors tried different antibiotics but none of the drugs were working...Yusra and her twin had been born two months premature... in northern Ethiopia where rebel and government forces are in violent conflict. Two years earlier, the hospital had been raided by a militia which stripped it of vital supplies. At six days old, Yusra’s sister died for want of a blood transfusion. ...Yusra’s mother took her on a five-hour journey, past numerous military checkpoints...Sepsis accounts for one in three newborn deaths in Ethiopia... her mother might not be able to afford it.”

This demonstrates how the structural burdens of AMR are, in fact, saliently highlighted, but fascinatingly, this anecdotal depth is not replicated in the western portrayals of AMR. and when it is, it looks something like this:

“This patient was a 62-year-old with a history of HIV and hepatitis C, both of which may have left him immunocompromised and unable to fight off the infection on his own. He had survived shigella twice prior, as well as Mpox, syphilis, and gonorrhea.”

We can see this patient is described as elderly, immunocompromised, and infected with one of the most stigmatizing diseases- HIV, having previously had STD’s which are also stigmatized and allude to homosexuality, you guessed it, also stigmatized. So when the anecdotal depth is replicated in western contexts, and even in non-western contexts, we see that the language used solely highlights the burden of AMR on the most vulnerable of our society rather than comprehensively highlighting it as something that affects us all.

This rhetoric is a defense mechanism: Even in materials ostensibly meant to raise awareness of the way AMR impacts the broader public, we distance ourselves from infections because it makes us more comfortable— I don't have HIV or live in war-torn Ethiopia, so this would never happen to me. Even though, it actively is happening, to everyone. Moreover, we are eclipsing our collective future vulnerabilities with these narratives of present vulnerability, as this problem, no matter where it comes from, continues to grow in ferocity and impact us all.

Simultaneously, we are creating a situation that adheres to outdated contain and control infectious disease efforts, instead of speaking to the broader structural causes of AMR and infectious diseases: such as overuse in agriculture, overprescription by doctors, a lack of regulatory intervention, in western and non-western localities alike. Indeed, our response to infectious disease is to isolate, to quarantine. Social distancing has its role, but studies now show that our instincts to close down and strictly limit travel to foreign countries was entirely unhelpful in limiting the spread of COVID: in a globalized world, disease always gets through the cracks. What it did do, was cost us vital time to prepare by gathering resources and PPE. Our instinct towards stigma and isolation must be interrogated: It is vital to how we tackle the next infectious disease crises we will face, from antimicrobial resistance and beyond.

CONFERENCE PAPER SLIDES:

https://docs.google.com/presentation/d/1tvQju8HkfeICvGbJey2Lr_bcRSz1RWJI/edit?usp=sharing&ouid=113470225056633485270&rtpof=true&sd=true

